

1. Administrative & Study Identification

Full Study Title: A PHASE 2, MULTICENTER, RANDOMIZED, DOUBLE-BLIND, PLACEBO-CONTROLLED, 16-WEEK STUDY EVALUATING THE SAFETY AND EFFICACY OF RITLECITINIB (PF-06651600) IN ADULTS WITH MODERATE TO SEVERE HIDRADENITIS SUPPURATIVA **Short Title/Acronym:** Ritlecitinib in Adults With Hidradenitis Suppurativa (HS)
Protocol Number: PF-06651600 (Investigational Product Code) **NCT Number:** NCT07228390 **Trial Status:** Not Yet Recruiting **Sponsor/Company Name:** Pfizer **Investigational Product:** Ritlecitinib / Trade Name: Litfulo (Oral JAK3/TEC inhibitor) **Phase of Development:** Phase 2 **Medical Monitor:** Pfizer Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety and efficacy of the study medicine (ritlecitinib) compared to a placebo in adults with moderate to severe Hidradenitis Suppurativa (HS) based on the proportion of responders achieving at least a 50% reduction in Hidradenitis Suppurativa Clinical Response (HiSCR50) at Week 16. **Secondary Objectives:** To assess the proportion of participants achieving HiSCR50, HiSCR75, and HiSCR90 response in patients treated with ritlecitinib versus placebo.

2.2 Background & Rationale Hidradenitis suppurativa (HS) is a debilitating skin disease characterized by long-lasting, painful, red skin lumps. Ritlecitinib is a small-molecule targeted therapy acting as an oral JAK3/TEC inhibitor, designed to target specific pathways in the immune system to reduce inflammation. This trial aims to generate safety and efficacy data for ritlecitinib in patients who have had an inadequate response to standard oral antibiotics.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** Randomized (Parallel Assignment) **Trial Status:** Not Yet Recruiting

3.2 Planned Enrollment & Duration Target \$N\$: 240 evaluable patients **Number of Sites:** Multicenter (including multiple sites in the United States) **Estimated Study Start:** 13-Nov-2025 **Estimated Primary Completion:** 05-Mar-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Male or female participants ≥ 18 to ≤ 75 years of age. 2. Diagnosis (based on clinical history and physical examination) of moderate to severe HS for at least 6 months prior to the Screening Visit, corresponding to Hurley stage II or III, with ≥ 5 HS lesions. 3. Inadequate response to at least a 4-week (28 days) treatment course with oral antibiotics for the treatment of HS.

4.2 Exclusion Criteria 1. Presence of ≥ 20 draining fistulae at Screening or Baseline (Day 1) visit. 2. Prior exposure to any JAK or BTK inhibitor, or having a known immunodeficiency disorder. 3. History of systemic infection requiring hospitalization or parenteral therapy, including history of infection with Mycobacterium TB. 4. Specific viral infection history (including history of herpes zoster, Hepatitis B (HBV), or Hepatitis C (HCV) Infection), or evidence of other active skin diseases or conditions at screening. 5. Current or recent history of clinically significant severe, progressive, or uncontrolled other medical conditions. 6. Any medical or psychiatric condition including any active suicidal ideation in the past year or suicidal behavior in the past 5 years.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Once daily (QD). Participants will start with a loading dose of ritlecitinib (or placebo) for 8 weeks, followed by a maintenance dose for an additional 8 weeks. **Route of Administration:** Oral **Duration of Treatment:** 16 weeks of active treatment, with the total study observation lasting about 24 weeks (6 months).

5.2 Concomitant & Prohibited Therapy Permitted: Standard non-pharmacological HS management regimens and stable concomitant therapies not interacting with JAK3/TEC pathways. **Prohibited:** Use of JAK or BTK inhibitors, other investigational drugs, and oral antibiotics for HS outside of the permitted protocol boundaries.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Physical Exam, Baseline Diagnostics
Baseline	Day 1	Randomization, First Dose, Vitals, Baseline HS assessment
Interim	Weeks 1, 2, 4, 6, 8, 12	Clinic visits including physical exams, blood/urine tests, ECGs, chest X-rays, hearing tests, and questionnaires
End of Study	Week 16 (to Week 24)	Final active-treatment Efficacy Assessment (HiSCR50), Exit Labs, 8-week Safety Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Difference in the proportion of responders based on Hidradenitis Suppurativa Clinical Response achieving at least a 50% reduction from baseline (HiSCR50) in patients treated with ritlecitinib versus placebo at Week 16.

Measurement Method: Clinical lesion counting and investigator-assessed physical examination tracking.

7.2 Secondary & Exploratory Endpoints - Proportion of participants achieving HiSCR50 response. - Proportion of participants achieving HiSCR75 response. - Proportion of participants achieving HiSCR90 response.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Method of collection includes physical exams, vital signs, chest X-rays, ECGs, hearing tests, and daily eDiary tracking on a mobile phone to monitor side effects and symptoms. **Serious Adverse Events (SAEs):** Captured throughout the 24-week period and reported per standardized regulatory timelines. **Data Monitoring Committee (DMC):** Internal and external safety board oversight facilitated by Pfizer.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy evaluation; Safety Set for all participants receiving at least one dose of the study drug. **Sample Size Justification:** $N=240$, scaled adequately to observe statistically significant variances in the primary HiSCR50 outcome between the ritlecitinib and placebo cohorts. **Handling of Missing Data:** Standard multiple imputation or Non-Responder Imputation (NRI) for binary primary endpoints (HiSCR50).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Combination of scheduled onsite clinic visits (approximately 10 total) and continuous compliance monitoring via patient eDiaries. **Audit Trail:** Robust clinical tracking via centralized EDC (Electronic Data Capture) systems for laboratory, imaging, and eDiary data.

11. Study Identifiers & Governance

Primary ID: PF-06651600 **Posting ID:** 965072330046414301 **Registry Number:** NCT07228390 **Sponsor/Lead Organization:** Pfizer **Study Title:** A 16-Week Study to Learn About the Study Medicine Called Ritlecitinib in Adults With Long Lasting Painful Red Skin Lumps, Known by the Medical Term, Hidradenitis Suppurativa, or HS. **Official Regulatory Title:** A PHASE 2, MULTICENTER, RANDOMIZED, DOUBLE-BLIND, PLACEBO-CONTROLLED, 16-WEEK STUDY EVALUATING THE SAFETY AND EFFICACY OF RITLECITINIB (PF-06651600) IN ADULTS WITH MODERATE TO SEVERE HIDRADENITIS SUPPURATIVA.

12. Clinical Framework (Hidradenitis Suppurativa Specific)

Primary Condition: Hidradenitis Suppurativa (HS) **Diagnosis Threshold:** Moderate to severe HS, Hurley stage II or III, with ≥ 5 HS lesions. **Medical Methodology:** Targeted Immunotherapy (JAK3/TEC inhibition) **Optimization Target:** $\geq 50\%$ reduction in HS lesions from baseline (HiSCR50).

13. Study Procedures & Methodology

Management Pathway: 8 weeks of loading dose followed by 8 weeks of maintenance dosing. **Education Protocol (HCP):** Protocol training, rigorous endpoint tracking calibration (HiSCR50 criteria). **Education Protocol (Patient):** Daily symptom and medicine adherence self-reporting via a mobile eDiary system. **Quality Audit Mechanism:** Reconciling drug dispensing records with mobile eDiary entries, alongside continuous laboratory, audiometry, and ECG evaluations.

14. Observation & Follow-Up Schedule

Total Duration: ~24 weeks (16-week intervention + 8-week safety follow-up). **Onsite Visit Cadence:** Screening, Day 1, and then every 1, 2, or 4 weeks until Week 16 (approximately 10 clinic visits). **Remote Monitoring Frequency:** Daily adherence and symptom tracking (eDiary). **Checklist Review Interval:** Routine investigator evaluations at the scheduled clinic visits.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead	
Statistician	[Name]	_____	[DD-MMM-YYYY]				